

Name: احلام فندی رسلی متی
National ID: 27901012111907



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

احلام
فندي رسلبي منى
٨٩ ش محمد جمعة - حوى سليم - عمرانية غ
العمرانية - الجيزة

٢٧٩٠١٠١٢١١٩٠٧

١٩٧٩/٠١/٠١

KY9091287

٢٠٢٥/٠١/٠١ ٢٧٨٠١٠١٢١١٩٠٧

بدون عمل

أنثى
مسجلة
متروجة
عطاس الناصح عبد عوض
البطاقة مارية حتى ٢٠٢٢/٠١/٠١

٢٠٢٢/٠١/٠١



Presetia Program for Women's Health
Ministry of Health and Population

البرنامج الراسمي للصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: احلام قنديل رستم عشتي

Sex:

MDT form

3- Medical number:

83264035

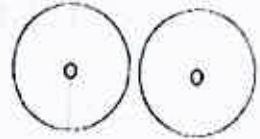
Old Med:

24 MAR

708-2025

2-ID number: 27901012111907

Patient Age: 46 Y



Family history

Family history
-ve

Phone

Phone
01278135773

BMI

AM

Marital status

Marital status
-ve

NRS Score

NRS Score

GROUP

GA 3

Co morbidity

Co morbidity
-ve

Menopausal
status

Menopausal status
PERIMENOPAUSE

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT
BREAST CANCER

Date of MDT

Date of MDT
2025-05-15

Breast Imaging:

It breast UIQ suspicious area of distortion with EXTENSION BIRADS 4

Clinical data

Mets Workup:
cap:free

BX

IDC

ER

8/8

PR

8/8

Her 2 Neu

0

SISH

Ki67

10

T

2M

N

0

M

0

Sonomammography

LEFT UIQ EXTENDING TO UOQ STARTING AT 11 TILL 12 , OTHER LESION AT 1-2 OCLOCK

PATIENT HISTORY

ON REVIEWING MAMMO MICROCALCIFICATIONS EXTENDING IN MULTIPLE QUADRANTS
upfront SM+SLNB: PT2(2.2CM)free deep margin
LN 0/6

+ve microcalification,ANGIOINVASION

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

ADJUVANT TAXOTERE ENDOXAN X4
ZOLADEX A1

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

Signature

SURGERY

SURGERY

Signature

Psychiatric

Psychiatric

Signature

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

Signature

PATHOLOGY

Signature

Nutrition

Signature

Name: <u>احلام افندي رسلي مني</u>		
MR. : <u>4632</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presented for continuation of letrozole (Femara) therapy and was provided with supportive treatment for a three-month period, with the next appointment scheduled for 31 January 2026. The patient was also cleared for a dental consultation prior to receiving zoledronic acid (Zometa). The patient was identified as having breast cancer and was placed on adjuvant endocrine therapy with an aromatase inhibitor (AI) and zoledronic acid (Zoladex) initiated in August 2025, with the AI planned for a three-month course in conjunction with zoledronic acid. The patient continued on letrozole (Femara) combined with zoledronic acid administered on 24 August 2026, along with supportive care for three months; the next follow-up was scheduled for 4 November 2026. The patient remains on letrozole (Femara) together with zoledronic acid, with supportive therapy planned for three months and the next visit set for 20 November 2026.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : <u>نهى سعيد علي محمد</u>	
يعتمد ,		



وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
قسم الأورام

Echo report

Name: Ahlam Fendy
Date: 13/4/2025

age: 46Y

V.A
CO

EF	63%	54-74%	PWd	0.9	0.6-1.1cm
LVEDD	4.0	3.5-5.7cm	AO	2.4	2-3.7cm
5LVESD	2.6	2-4cm	LA	3.3	1.9-4cm
FS	34	25-45%	RV		0.7-2.3cm(supine)
IVSd	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

Normal LV internal dimensions and systolic function (EF=63%) by M-mode.

Normal mitral inflow pattern.

No resting SWMA could be detected.

Normal LA size.

Mitral Valve: Normal valve and flow.

Aortic valve: Normal valve and flow.

Tricuspid valve: Normal valve and flow.

Right side and systolic function: Normal, TAPSI= 23 mm.

No intracardiac masses or thrombi.

Normal pericardium with no pericardial effusion.

Normal IVC diameter and collapsibility.

Conclusion:

Normal echo-Doppler study.

Dr Eman Kassem, MD
Cardiology



Patient Name: Ahlam Fndi Rslil Matta	Date : 04/03/2025
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 708/2025	Pat. ID: 98197

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

- Reported case of left breast cancer.

I Chest:

- Left breast mass lesion with irregular margin measuring about 19x 18 mm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is mildly enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- Reported case of left breast cancer revealed:
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Prof. Menna Talla Hanafy, M.D.



Ministry of Health and Social Welfare
Kingdom of Saudi Arabia

البرامج الرئيسية لخدمة المرأة
وزارة الصحة والسكان



A-Patient ID:
1- Name: احلام قنديل رسلبي مثنى
2-ID number: 27901012111907

Sex:

MDT form
3- Medical number:

Female

83264035
Old Med:

708-2025

Patient Age: 45 Y

Family history

Family history
-VE

Phone

Phone

0177511577

BMI

BMI

Marital status

Marital status
-VE

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co morbidity
-VE

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

REASON OF MDT

Date of MDT

2025-05-27

Breast Imaging:

It breast UIQ suspicious area of distortion with EXTENSION IN RAD 1

BX
IDC

EX COMMENT

ER
8/8

EX COMMENT

PR
8/8

EX COMMENT

T

2M

N

0

Clinical data

Mets Workup:
cap:free

Her 2 Neu
0

EX 2 Neu COMMENT

SISH

SISH COMMENT

KIG2
10

M

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ON REVIEWING MAMMO MICROCALCIFICATIONS EXTENDING IN MULTIPLE QUADRANTS

MDT OUTCOME & PLAN

FOR UPFRONT LEFT SIMPLE MASTECTOMY, SLNB

MEDICAL ONCOLOGY

SURGERY

Psychiatric

MEDICAL ONCOLOGY

Signature

Signature

Signature

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

Signature

Signature

Signature



Patient Name : الأستاذة / احلام فندى رسللى متى	Request No. : 13006470
Patient No. : 27901012111907	Request Date : 23/01/2025
Path No. : 321/25	Verification Date : 12/02/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 46 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Printing Date : 22/02/2025 09:36:56

Page 2 of 2

Printing By : .marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / احلام فتدي رسللى متى
Patient No. : 27901012111907
Path No. : 1314/25
Sex : Female
Age : 46 Y
Referring Doc :

Request No. : 13006944
Request Date : 14/04/2025
Verification Date : 29/04/2025
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

E-cadherin: Positive

Diagnosis:

- Left Simple mastectomy & SLND, (Upfront) :
Invasive duct carcinoma Grade II .
• Free LNs (0/6)
• Free nipple , free margin
• Positive microcalcification
• Stage : pT2 , pN0

Prof .Dr. Gamal Abdeatty
Dr.Nermin Mohamed

Verified by :
Dr.Heba Hasan

Printing Date : 04/05/2025 12:09:00

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



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age: 46Y

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CO

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Cardiology



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- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- Reported case of left breast cancer revealed:
- No sizable pulmonary or hepatic deposits on non-contrast basis.

Prof. Menna Tallah Hanafy, M.D.

Name: Ahlam findi .
Date: 16-1-2025

Women's Health Outreach Program

Clinical summary: Bilateral mastalgia
Technique Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered dense breast parenchyma (ACR density B).
- An Area of architectural distortion seen in left middle 1/5 of left UIQ extending to the UOQ with group of amorphous calcifications extending to 6 cm lower zone B confirmed by US to be an irregular shaped hypoechoic ill defined solid mass lesion starting at 11 :00 till 12 , at 7 mm from skin surface at 5 cm from nipple , it is seen measuring 3.3 x 2.8 x 2 cm in average dimensions , another adjacent hypoechoic cystic lesion at a distance of 1 cm from the latter measuring around 8 x 6 mm, both show vascularity on color Doppler interrogation .
- Left axillary showed non specific lymphnodes with preserved shape & central echogenic fatty hilum yet minimal focal cortical thickening 2.5 mm.
- At 1-2 o'clock position , another smaller hypoechoic lesion seen 0.4cm likely satellite .
- Bilateral inverted nipples yet normal skin .
- No supra or infraclavicular lymphnodes .

Right breast

- Scattered mixed dense breast parenchyma (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Normal skin/subcutaneous tissue and inverted nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Left breast (BIRADS 4): left UIQ suspicious area of architectural distortion with extensions ?? satellites , for histopathological verification & biopsy .

Left breast (BIRADS 1): Unremarkable study .

Best regards,
Dr. Sarah Hussein & Dr.
hedaya Nour



Patient Name	: الأستاذ / احلام فندى رسللى متى	Request No.	: 13006470
Patient No.	: 27901012111907	Request Date	: 23/01/2025
Path No.	: 321/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left UOQ breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 23/1/2025 at 9.40 am.

Gross Examination:

- 2 grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
 - PR: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
 - Her2: No membrane reaction in the tumor cells (score 0).
 - Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Left breast UOQ lesion, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 8 /8).

Verified by :

DR.Menna Nabil

Printing Date : 22/02/2025 09:36:56

Page 1 of 2

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Patient Name : الأستاذة / احلام فندى رسللى متى
Patient No. : 27901012111907
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Sex : Female
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Request No. : 13006944
Request Date : 14/04/2025
Verification Date : 29/04/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data : Left breast cancer(Up front)

Nature of the specimen : Left simple mastectomy &SLND

Gross Examination:

• Procedure: Left simple mastectomy specimen measured 22x19x4 cm , covering skin ellipse measured 22x12 cm .

• Tumor extent : An ill defined greyish white firm mass measured 2.2x1.5 cm .

• Distance from margins: 2cm away from deep margin .

• Lymph nodes number and size: A fibrofatty tissue piece measured 5x3x2 cm enclosing 6 SLNs up to 1.5 cm .

Frozen section : Free SLNB(0/6)

Block designation:

1,2 .mass

3,4 random

5.nipple

6.deep

7,8.SLNB

Grossing by : Dr.Nermin Mohamed

Microscopic examination:

• Histological type: Invasive duct carcinoma

• Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)

• Angioinvasion : Positive

• Microcalcification : Positive

• Perineural invasion : No

• Ductal carcinoma in situ: No

• Lymph node involvement: Free (0/6)

• Resection Margins: Free deep margin

• Additional pathologic findings: Positive microcalcification

IPT:

Verified by :

Dr.Heba Hasan

Printing Date : 04/05/2025 12:09:00

Page 1 of 2

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CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مصر

MDT FORM

18/3/2025 G

Name	اسلام هادي راسلا	MRN	
Age	50	ID	
Menopausal status	Peri	Phone	01011405774
NRS SCORE		Marital status	Married
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	

Clinical Data

BX: IDC 92

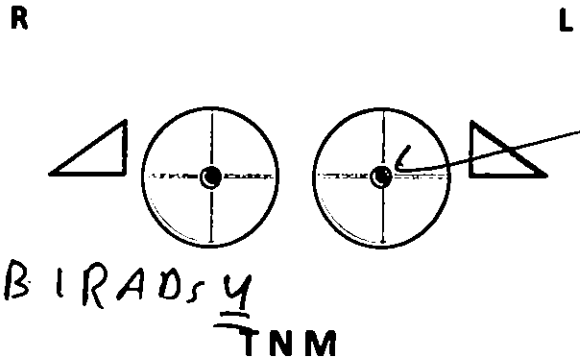
ER: P/8 PR: D/8 HER 2: -ve KI67: 100%

Sonomammography:

Left breast U/A suspicious
area of distortion with extensive

Mets Workup:

CT - chest Abd
- pelvis - free



BIRADS 4

TNM

Recommendations:

دكتور
13/11
2025

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY SURGERY RADIOLOGY PATHOLOGY Nutrition Psychiatric



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : احلام قنديل رسلبي متى

Sex:

3- Medical number:

Female

83264035

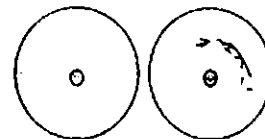
Old Med :

Old Med

708-2025

2-ID number: 27901012111907

Patient Age : 46 Y



Family history

Family history
-ve

Phone

Phone

01278135773

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP
6

Date of MDT

Date Of MDT

2025-03-27

Co morbidity

Co morbidity
-ve

Menopausal
status

Menopausal status
PERIMENOPAUSE

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

lt breast UIQ suspicious area of distortion with EXTENSION BIRADS 4

Mets Workup :

cap:free

BX IDC BX COMMENT	ER 8/8 ER COMMENT	PR 8/8 PR COMMENT:	Her 2 Neu 0 Her 2 Neu COMMENT	SISH SISH COMMENT :	Ki67 10
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T 2M	N 0	M 0
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Sonomammography

LEFT UIQ EXTENDING TO UOQ STARTING AT 11 TILL 12 , OTHER LESION AT 1-2 OCLOCK

PATIENT HISTORY

ON REVIEWING MAMMO MICROCALCIFICATIONS EXTENDING IN MULTIPLE QUADRANTS

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR UPFRONT LEFT SIMPLE MASTECTOMY, SLNB

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY / د. علاء جليل	RADIOLOGY	RADIOLOGY / د. محمد تاجي
SURGERY	SURGERY / د. بهادر حسن	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



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1- Name : احلام قنديل رسلبي متى

Sex:

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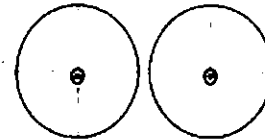
Old Med :

Old Med :

708-2025

2-ID number: 27901012111907

Patient Age : 46 Y



Family history

Family History
-ve

Phone

Phone
01278135773

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP
B

Date of MDT

Date Of MDT
2025-05-15

Co morbidity

Co Morbidity
-ve

Menopausal
status

Menopausal Status
PERIMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

It breast UIQ suspicious area of distortion with EXTENSION BIRADS 4

Mets Workup :

cap:free

BX IDC BX COMMENT	ER 8/8 ER COMMENT	PR 8/8 PR COMMENT	Her 2 Neu 0 Her 2 Neu COMMENT	SISH SISH COMMENT :	Ki67 10
-------------------------	-------------------------	-------------------------	-------------------------------------	------------------------	------------

T 2M	N 0	M 0
---------	--------	--------

Sonomammography

LEFT UIQ EXTENDING TO UOQ STARTING AT 11 TILL 12 , OTHER LESION AT 1-2 OCLOCK

PATIENT HISTORY

ON REVIEWING MAMMO MICROCALCIFICATIONS EXTENDING IN MULTIPLE QUADRANTS

upfront SM+SLNB: PT2(2.2CM)free deep margin

LN 0/6

+ve microcalification,ANGIOINVASION

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

ADJUVANT TAXOTERE ENDOXAN X4

ZOLADEX AI

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY / د. علاء جميل	RADIOLOGY	RADIOLOGY / د. رانيا البشري
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



وزارة الصحة والمساكن
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام
قسم الايكو

Echo report

Name: Ahlam Fendy

Date: 13/4/2025

age: 46Y

4.1
CO

EF	63%	54-74%	PWd	0.9	0.6-1.1cm
LVEDD	4.0	3.5-5.7cm	AO	2.4	2-3.7cm
5LVESD	2.6	2-4cm	LA	3.3	1.9-4cm
FS	34	25-45%	RV		0.7-2.3cm(supine)
IVSd	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

Normal LV internal dimensions and systolic function (EF=63%) by M-mode.

Normal mitral inflow pattern.

No resting SWMA could be detected.

Normal LA size.

Mitral Valve: Normal valve and flow.

Aortic valve: Normal valve and flow.

Tricuspid valve: Normal valve and flow.

Right side and systolic function: Normal, TAPSI= 23 mm.

No intracardiac masses or thrombi.

Normal pericardium with no pericardial effusion.

Normal IVC diameter and collapsibility.

Conclusion:

Normal echo-Doppler study.

Dr Eman Kassem, MD
Cardiology

رقم الملف:



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إدارة الجودة

Doctor's Orders **اوامر الطبيب**

.....: نوع التغذية:: الشخص:

[illegible]

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

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التشخيص:..... **نوع التغذية:.....**

التاريخ	الوقت	الأوامر	توقيع الطبيب
		1- علاج الكوبدولون + دوكس + 14 2- تاكسيما 1000 (1000) / 14 3- الستيروئيد - 1000 / 14 4- ميباموكس 1000 / 14 5- دوكسولون 1000 / 14 6- ريتونافير 1000 / 14 7- فافير 1000 / 14 8- سالفون 1000 / 14 9- بروفين 1000 / 14 10- جانتافير 1000 / 14 11- جانتافير 1000 / 14 12- جانتافير 1000 / 14	19-7-2023

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

4/16/7